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/ **Practice Exam - 1**

Correct Answer

Next Question

Practice Exam - 1

Question 71 of 117



A 63-year-old male with a history of diabetes and coronary artery disease who underwent implantation of a HeartMate 3 LVAD 6 months ago presents in the office with shortness of breath and fatigue with minimal exertion. He is on aspirin 325mg daily and coumadin 3 mg daily. On exam BP is 124/90 mmHg (MAP 101mmHg) measured by an automated cuff. HR is 84 bpm. The JVP is just above the clavicle with the patient sitting upright and there is 1+ edema. LVAD Speed is 6000 RPM with a flow of 3.2 L/min and Power of 4.5 Watts. No LVAD alarms are noted. Labs are notable for a Hct of 36%, a creatinine of 1.1 mg/dl and potassium of 4.1mmol/L.

Which of the following should be recommended next?

- A. Refer for hemodynamic "Ramp" study
- ☒ B. Initiation of Lisinopril 5 mg daily
- C. Perform a CT scan of the chest with contrast
- D. Refer for Cardiac Rehabilitation

Commentary

References

Testing Point: The Board-certified AHFTC specialist should realize the importance of optimal blood pressure management in LVAD patients.

Answer: B. Initiation of Lisinopril 5mg daily.

Hypertension is a common cause of heart failure post LVAD. The Ideal MAP in LVAD patients is between 65-80 mmHg. This patient clearly has an elevated MAP which is decreasing his cardiac flow and is likely contributing to his heart failure symptoms. Increasing evidence specifically suggests benefit of ACE inhibitors in LVAD patients. Though further prospective study is required to confirm the benefit of ACE inhibitors, in this case they would be preferable to hydralazine given their once daily dosing and the history of diabetes and coronary disease.

Answer D: Though cardiac rehabilitation is beneficial in LVAD patients, optimizing blood pressure should be done prior if possible.

Answer A: A RAMP study would be indicated if symptoms persisted despite bp control.

Answer C: The pump appears to be functioning normally. While a chest CT may be useful in cases of suspected pump thrombosis or dysfunction, there is no evidence of that complication here.

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